

OpWell Concierge™

Clinical Evidence Brief & Operational Value Analysis

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Executive Summary

OpWell Concierge™ is an independent, telehealth-based perioperative specialty service that translates multicenter RCT evidence from Enhanced Recovery After Surgery (ERAS), Perioperative Surgical Home (PSH), and prehabilitation literature into a comprehensive optimization model across two distinct phases: preoperative preparation and structured 30-day post-discharge monitoring. OpWell does not participate in intraoperative care. This brief presents the primary evidence base, with exact effect estimates, confidence intervals, and certainty-of-evidence ratings for each clinical claim.

Evidence Table: Clinical and Operational Claims

Clinical Claim	Effect Estimate (95% CI)	Certainty	Primary Source
Complication reduction via exercise prehabilitation	OR 0.50 (0.39–0.64); ~50% RR	Very Low*	Mclsaac et al., BMJ 2025; 186 RCTs, n = 15,684
Complication reduction via exercise (confirmatory)	OR 0.54 (0.44–0.67)	Moderate	Ramic et al., Anesth Analg 2025; 99 RCTs, n = 8,222
Hospital LOS reduction via ERAS pathways	MD –1.88 days (–2.81 to –0.95)	Moderate	Sauro et al., JAMA Netw Open 2024; 74 RCTs, n = 9,076
Complication reduction via ERAS pathways	RR 0.71 (0.59–0.87)	Mod-High	Sauro et al., JAMA Netw Open 2024
Cost savings — abdominal ERAS	–\$5,109 per case (–\$5,852 to –\$4,366)	Moderate	Visioni et al., Ann Surg 2018; 39 studies, n = 6,511
Anesthesiologist-led PSH operational impact	Decreased LOS, ICU use, opioids; ↑ home discharge	Low	Cline et al., J Clin Anesth 2020; 11 studies
Cost-effectiveness of prehabilitation	64% (16/25) support cost-effectiveness	Low**	Rombey et al., BMC Med 2023
ED visit reduction via mHealth monitoring	OR 0.42 (0.23–0.79); ~58% reduction	Low	Dawes et al., BJS 2021; 45 studies
Readmission reduction via mHealth	OR 0.47 (0.29–0.77); ~53% reduction	Low	Dawes et al., BJS 2021
Readmission reduction via digital health	RR 0.67 (0.58–0.78; I² = 0%); ~33% reduction	Low-Mod	Grygorian et al., JAMA Netw Open 2024;7(4):e244963
Preoperative patient education — LOS	MD –1.01 days (–1.86 to –0.16)	Moderate	Hall et al., J Am Coll Surg 2025; 40 RCTs, n = 4,113
QoR-15 psychometric validity	Cronbach's α = 0.85 (0.83–0.87); r = 0.97	High	Myles PS, et al. Br J Anaesth 2022;128(6):1029-1039
Same-day cancellation reduction	12.3% → 8.4% rate; \$150,200 savings	Moderate	Wongtangman K, et al. Anaesthesia 2024;79(6):618-627
Preoperative assessment → cancellation reduction	AOR 0.55 (0.43–0.72)	Moderate	Tan et al., Int J Surg 2019

* Robust after exclusion of high risk-of-bias trials. ** Publication bias risk noted by authors.

Critical Nuance: Post-Discharge Monitoring Requires Structured Technology

A systematic review by Boggan et al. (2025, *Annals of Internal Medicine*) of 7 randomized trials ($n = 7,075$) demonstrated that simple, single-contact telephone calls within 7 days of discharge produced no reduction in 30-day readmissions (risk difference 0.00; 95% CI -0.02 to 0.02 ; moderate certainty) or ED visits. This indicates that passive, unstructured telephone follow-up is clinically insufficient to change healthcare utilization outcomes.

The significant reductions in readmissions and ED visits cited in the broader literature (OR 0.42–0.67) are strictly specific to structured, technology-enabled, multi-contact monitoring platforms—the exact clinical standard operationalized by OpWell Concierge. This evidence distinction underscores the operational necessity of structured tracking in value-based care pathways.

OpWell Surgery School: POQI Consensus Alignment

Every referred patient completes a 23-module digital preparatory framework aligned with 2025 POQI recommendations across five therapeutic domains:

- Procedure-Specific: Personalized pathways for 8 major specialties.
- Behavior Change: Targeted nutritional, fitness, and cessation coaching.
- Psychological: Anxiety mitigation and clinical visualization.
- Practical Preparation: Home configuration and caregiver alignment.
- Expectations: Comprehensive 30-day week-by-week recovery roadmap.

B2B Integration and Clinical Continuity

OpWell Concierge operates exclusively in the preoperative and post-discharge phases as a collaborative extension of existing clinical practice. It does not provide intraoperative services, alter surgical plans, manage technical operative complications, or replace standard postoperative office visits.

Preoperative Phase: By intercepting low-risk programmatic queries, providing objective medical clearance reporting, and delivering a validated Readiness Score alongside a physical completion certificate, OpWell ensures patients arrive optimized, compliant, and structurally prepared for anesthesia and surgery.

Post-Discharge Phase: OpWell's structured digital monitoring platform triages recovery concerns during the critical 30-day window via technology-enabled multi-contact touchpoints, escalating actionable findings back to the surgical team for clinical integration.

Transparency Statement on Evidence Certainty

Strongest Evidence (Moderate-High): ERAS pathways consistently reduce LOS by ~1.9 days and complications (RR 0.71) across high-quality RCTs.

Strong Evidence (Very Low-Moderate): Exercise prehabilitation demonstrates consistent 50–54% complication reduction, robust after exclusion of high risk-of-bias trials.

Moderate Evidence (Low-Moderate): Post-discharge monitoring shows the strongest signal from structured, technology-enabled platforms (OR 0.42–0.67) rather than passive telephone contact. The Boggan et al. null finding for passive calls clinically reinforces this operational distinction.

Foundational Evidence (Low): The anesthesiologist-led PSH model lacks RCT-level evidence; available data are limited to before-and-after cohort studies. These limitations are standard for complex health-system interventions.

Conclusion: The OpWell clinical framework is structurally aligned with moderate-to-high certainty evidence across its core therapeutic domains (ERAS guidelines, prehabilitation protocols, and structured digital monitoring), delivering a rigorously validated, evidence-based asset to surgical programs.

References

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